

ENGLISH FOR WORK / SEPTEMBER 2026

# Medical Devices English



## Vocabulary & phrasebook

Keep precise meanings and useful expressions close. Retrieve the words, then use them in a complete message.

### PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For medical device engineers, product managers, quality and regulatory staff, clinical specialists, manufacturing teams, field-service teams, and device-company leaders.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

# Make vocabulary usable

Knowing a definition is a start. To use a term well, explain it in plain English, connect it to a case, and choose a natural sentence around it.

- Read five terms, cover their definitions, and recall the meanings.
- Sort a pair you might confuse and explain the difference.
- Use one term in a sentence about a fictional case.
- Ask a partner to explain the sentence without repeating the specialist word.
- Return to the same terms on another day and test recall again.

## Abbreviations need context

Expand an abbreviation on first use when the listener needs it. Some abbreviations have different meanings across professions: API can refer to an application programming interface or an active pharmaceutical ingredient. The course context determines the meaning.

## Definitions have limits

These are concise language-learning explanations, not complete legal, clinical, or technical specifications. Use the applicable authoritative definition for regulated or operational decisions.

# Field vocabulary A-Z

## **510(k)**

US FDA premarket submission that typically demonstrates substantial equivalence to a legally marketed predicate device.

## **acceptance criteria**

Specific conditions a deliverable must meet to be accepted.

## **clinical evidence**

Information from clinical evaluation or research relevant to a health-related question.

## **complaint**

An expression of dissatisfaction requiring attention under a relevant process.

## **De Novo**

A US FDA classification process for certain novel devices without a legally marketed predicate.

## **design input**

A documented requirement that guides a product's design.

## **design review**

A structured evaluation of a design at a defined stage.

## **FMEA**

Failure mode and effects analysis, a structured method for identifying how a process or product can fail and prioritizing controls.

## **formative study**

An evaluation used during development to identify problems and improve a design.

## **harm**

Injury, damage, or another adverse effect on people, property, or the environment.

## **hazard**

A potential source of harm.

## **human factors**

The study and application of knowledge about how people interact with products, tasks, and environments.

## **instructions for use**

The manufacturer's information explaining the intended use and operation of a device.

**intended use**

The purpose for which a product is designed and represented to be used.

**labeling**

The information supplied with or about a regulated product, as defined by applicable rules.

**lot traceability**

Ability to follow a production lot through materials, process steps, tools, inspections, and downstream customers or products.

**malfunction**

A failure of a product or system to operate as intended.

**MDR**

Medical Device Reporting: the US FDA system for specified device-event reports; elsewhere the acronym may refer to a regulation.

**nonconformance**

Failure to meet a requirement, specification, contract, standard, or approved procedure.

**off-label**

A use outside a regulated product's approved or cleared labeling, as applicable to that product and jurisdiction.

**PMA**

Premarket approval: the US FDA review pathway for certain high-risk medical devices.

**postmarket surveillance**

Collecting and evaluating information about a product after it enters the market.

**predicate device**

A legally marketed device used as a comparison in an applicable US FDA submission.

**process validation**

Documented evidence that a process can consistently produce results meeting specified requirements.

**residual risk**

The risk remaining after controls or other responses are applied.

**summative study**

An evaluation conducted to assess performance against defined requirements after development work.

**supplier CAPA**

A supplier's corrective and preventive action process for addressing identified quality problems and recurrence risks.

**traceability**

The ability to follow an item's history, source, requirements, or records through documented links.

**use error**

An action or omission during use that leads to a different result from the intended one; context and design matter.

**user need**

Something a user needs to accomplish in a particular context.

**validation**

Confirming that an output or system meets needs for its intended use under defined conditions.

**verification**

Checking whether specified requirements have been met.

# Expressions for this course

Complete the frames with the facts of your case. A frame helps organize meaning; it should not replace an actual explanation.

## Ask a precise question

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is ... . Is that right?

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

## Make a complex point clear

In this context, ... means ... .

The difference is that ... .

How would you explain that distinction to a colleague?

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

## Explain a fair comparison

Both options ..., but ... .

Option A ..., whereas option B ... .

The comparison depends on ... .

Use the same time period, scope, and measurement basis for both options. Explain a difference with 'whereas' or 'while'. State what improves and what is given up; do not hide the weaker side of a recommendation.

## Match certainty to evidence

The available evidence shows ... .

This may indicate ..., but ... .

We have not yet established whether ... .

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

## Ask for an actionable next step

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know ... .

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

## Transfer information and responsibility

The current status is ...; the outstanding item is ... .

The record shows ... .

Please confirm who has accepted ... .

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

## Give a useful status update

We have completed ...; ... remains open.

As of ..., the status is ... .

The next update will cover ... .

Lead with the current state, then the important change or open item. Use completed-action language only for completed work. Add a next update point when it is known; do not invent a date to make the message sound complete.

## Disagree with a useful reason

I understand the goal. My concern is ... .

That statement goes beyond ... .

Could we ... before confirming ...?

Acknowledge the goal without pretending agreement. Name the specific problem and its implication, then ask a focused question or propose a next step. Be direct when a safety or ethical boundary requires it.

# Model responses in context

Read the situation first. Cover the model, say your own response, and compare the two for meaning and tone.

## 01 | User Needs and Design Inputs

A design brief says the device should be "easy to use." It does not identify the intended users or use environment.

Who will use it, for which task, and in what environment? Let's turn "easy to use" into specific user needs and requirements that the team can review.

Try it again: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

AI extension: build and retrieve vocabulary from this case.

## 02 | Risk Management and Hazard Analysis

A risk discussion uses hazard and harm interchangeably. The team needs to describe the potential source of harm and the possible outcome separately.

Let's distinguish the hazard from the harm that could result. That will help us describe the sequence clearly and discuss the controls without treating the two terms as synonyms.

Try it again: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

AI extension: build and retrieve vocabulary from this case.

## 03 | Verification, Validation, and Design Review

A device team has checked design outputs against specifications but has not yet completed the planned evaluation of intended user needs.

The specification checks address verification. They do not by themselves answer every validation question about intended use. I'll report those activities separately and identify what remains open.

Try it again: Your listener focuses only on the lowest price or fastest date. Explain one other relevant comparison criterion.

AI extension: build and retrieve vocabulary from this case.

## 04 | Usability and Human Factors

Several participants misread a label in a formative usability study. A draft report describes them as careless.

The observed difficulty was reading the label correctly in this study. Let's describe the task, design, and use conditions, then investigate contributing factors without assuming carelessness.

Try it again: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

AI extension: build and retrieve vocabulary from this case.

## 05 | Regulatory Pathways and Submissions

A presentation refers to a proposed device pathway as settled, although the regulatory team is still evaluating options.

Please label the pathway as under evaluation and identify the questions for the regulatory team. We should distinguish a proposed approach from a confirmed regulatory conclusion.

Try it again: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

AI extension: build and retrieve vocabulary from this case.

## 06 | Complaints, MDRs, and Postmarket Signals

A complaint report describes a malfunction but lacks the device identifier and event date. The complaint-handling team needs the available facts promptly.

Here is the reported malfunction and the information currently available. The device identifier and event date are missing. Please confirm receipt and handle follow-up through the complaint process.

Try it again: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

AI extension: build and retrieve vocabulary from this case.

## 07 | Manufacturing, Suppliers, and Nonconformance

An incoming-material review has identified two nonconforming lots. The investigation is open and disposition has not been authorized.

Two lots have open nonconformance records. The investigation and disposition are still pending. I'll report their status without implying that release or rejection has already been approved.

Try it again: The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

AI extension: build and retrieve vocabulary from this case.

## 08 | Clinical Training and Labeling Boundaries

A trainer is asked to demonstrate a use beyond the materials approved for the session.

That request goes beyond the approved training material for this session. I can cover the authorized instructions and route the additional question to the appropriate team for review.

Try it again: The other person repeats the request more firmly. Restate the specific concern calmly and explain the appropriate next route.

AI extension: build and retrieve vocabulary from this case.

# Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

## Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

## Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

### Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

### Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

### FDA: De Novo classification

<https://www.fda.gov/medical-devices/premarket-submissions-selecting-and-preparing-correct-submission/de-novo-classification-request>

### FDA: Medical Device Reporting

<https://www.fda.gov/medical-devices/medical-device-safety/medical-device-reporting-mdr-how-report-medical-device-problems>

## Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

# Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

## Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Writing feedback begins with your draft. Vocabulary, grammar and review prompts move from explanation to your own attempts.

## Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

## Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

## Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-28. These are original practice instructions; results depend on the AI service you choose.

# Build vocabulary and collocations

## START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

## REFERENCE

Course: Medical Devices English

Professional audience: medical device engineers, product managers, quality and regulatory staff, clinical specialists, manufacturing teams, field-service teams, and device-company leaders

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Lesson 01: User Needs and Design Inputs

Original fictional case: A design brief says the device should be "easy to use." It does not identify the intended users or use environment.

Language workshop: Ask a precise question

Communication goal: Clarify missing information before responding.

Language guidance: Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

Useful frames (complete the gaps with case facts): When you say ..., do you mean ...? / Could you clarify which ...? / So, my understanding is ... . Is that right?

Vocabulary: user need: Something a user needs to accomplish in a particular context. / design input: A documented requirement that guides a product's design. / intended use: The purpose for which a product is designed and represented to be used. / traceability: The ability to follow an item's history, source, requirements, or records through documented links.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Writing task: Write a 70-110 word message for the person who needs to act on this case. State the purpose, preserve the relevant facts, and make the next action or unresolved question clear. Use a subject line and an appropriate opening. Do not invent a deadline, finding, or approval.

Optional second-round challenge: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Model for comparison AFTER my attempt, not a response to give me first: Who will use it, for which task, and in what environment? Let's turn "easy to use" into specific user needs and requirements that the team can review.

**END REFERENCE**

**TASK:** Vocabulary in use

1. Choose four useful terms from the reference. For each, give a plain-English meaning in this field, two natural word combinations (collocations), one realistic example and a likely misuse or contrast. Keep the examples consistent with the reference and avoid jargon that does not fit this occupation.
2. Add four related terms that would be useful in a different common situation in the same field. Label these as suggested extensions, explain how they connect to the work, and flag regional or organizational variation where relevant. Do not pad the list with synonyms nobody would use at work.
3. Start a retrieval round: give one short workplace sentence with a gap and a clear clue. Ask me to supply the best term and explain my choice. Stop and wait; do not reveal the answer or a completed sentence yet. Accept another term if its meaning and collocation work.
4. After my attempt, explain one useful distinction and ask me to write my own sentence. Wait, correct a genuine meaning or usage problem, then give the next retrieval question. Work through four questions one at a time.
5. Finish with a short handoff or message task using three terms, followed by two recall questions I can save for another day. Do not pretend to schedule a reminder.

**END PROMPT**

Copy this prompt online or choose another lesson and support level

# Test recall and transfer

## START PROMPT / COPY THROUGH END PROMPT

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Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

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Model for comparison AFTER my attempt, not a response to give me first: Who will use it, for which task, and in what environment? Let's turn "easy to use" into specific user needs and requirements that the team can review.

**END REFERENCE**

**TASK:** A short retrieval session

1. Tell me to close my notes. Plan six questions: two vocabulary-in-context items, one grammar or meaning contrast, one question about a confirmed versus unknown detail, one short spoken-style response and one new-situation transfer task. Use the supplied material as the answer reference, but do not repeat its existing checks word for word.
2. Ask only the first question, then stop and wait. Prefer a short answer over a multiple-choice question. Do not display the remaining questions, model answers or a word bank in advance.
3. After each answer, say what it gets right and explain a meaningful error if there is one. Accept alternative wording when it preserves the facts and does the communication job. If I struggle, give one clue and invite a retry before revealing a model. Only then move to the next question.
4. Keep a simple record of which items I answered independently, with a hint, or after seeing a model. This records today's practice, not a certified proficiency score. Do not judge pronunciation from typed text.
5. At the end, summarize two strengths and two useful next steps with examples from my answers. Give me a small review card containing three prompts and a separate answer section. Suggest that I revisit it tomorrow and a week later; do not claim you will remember this session or send reminders. Label the final transfer scenario as fictional.

**END PROMPT**

Copy this prompt online or choose another lesson and support level